



SRI LAKSHMI NARAYANA INSTITUTE OF MEDICAL SCIENCES

OSUDU, AGARAM VILLAGE, VILLIANUR COMMUNE, KUDAPAKKAM POST, PUDUCHERRY - 605 502.

DEPARTMENT OF GENERAL MEDICINE DISCHARGE SUMMARY

NAME	: [REDACTED]	UHID	: [REDACTED]
AGE/SEX	: [REDACTED]	IPNO	: [REDACTED]
ADDRESS	: 44, ELLAIAMMAN KOVIL ST VANARAPET		

D.O.A : 14/12/2024

D.O.D : 21 /12/2024

DIAGNOSIS:

1. CKD under MHD@ R(FEMORAL)
2. ACUTE PULMONARY EDEMA-(RESOLVED)
3. TYPE 2 DM
4. ANEMIA OF CHRONIC DISEASE

CHIEF COMPLAINTS:

62 yrs old female k/c/o T2DM, SHTN, CKD

Came with the complaints of breathlessness since yesterday night

HISTORY OF PRESENTING ILLNESS:

Patient was apparently normal until yesterday, afternoon she had undergone AV Fistula operation in left upper limb, later at night she had sudden onset breathlessness more on lying down, and increasing on sitting position

No h/o cough, cold, fever

No c/o chest pain, palpitation, giddiness

No h/o vomiting, abdominal pain, urinary complaints

PAST HISTORY

K/C/o T2DM and HTN since 7 yrs

Diabetic on inj MIXTARD 8-0-8

HTN on T. NILASDIA 10mg 1-1-1

PERSONAL HISTORY:

Mixed diet

Normal Bladder & Bowel habits,

Normal sleep pattern

ON EXAMINATION:

General Examination

Patient was conscious, oriented

Pallor (+), pedal edema (+)

No icterus / clubbing / cyanosis / lymphadenopathy

VITALS:

PR: 100/min

RR: 18 breaths / min

BP: 120/70mmHg

SpO2: 98%at RA

S/E:

CVS : S1,S2 (+)

RS : B/L NVBS(+)

P/A :Soft ,non tender

CNS :NFND

INVESTIGATIONS:

ABG (14/12/24)-

pH- 7.37

PCo2-27.7 mmHg

PO2- 122mmHg

Hco3-15.8 mmol/L

Blood group(14/12/24)- "O" Positive

Blood culture and sensitivity-(14/12/24)-

No growth after 48 hours of incubation

Serology	14//12/24
HIV	Non- reactive
HBsAg	Negative
HCV	Non- reactive

Urine routine	15/12/24
Urine albumin	2+
Urine sugar	1+
Pus cells	5-6
Epithelial cells	3-4



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CBC:	14/12/24	19/12/24
HB	6.2g/dL	7.0
RBC	$2.13 \times 10^6/\text{mm}^3$	2.50
PCV	17.9%	20.3
MCV	84fL	81.2
MCH	28.9pg	28.1
MCHC	34.4g/dL	34.7
PLT	$309 \times 10^3/\text{mm}^3$	347
WBC	$5.89 \times 10^3/\text{mm}^3$	13.84
N	5.20%	55.9
L	88.6	25.7
M	9.1	16.1
B	0.4	0.6
E	0.3	2.1

RP2	14/12/24	15/12/24	16/12/24	19/12/24
Urea	78.9mg/dL		73.0mg/dL	60.9 mg/dL
Creatinine	6.17mg/dL		7.25mg/dL	5.26 mg/dL
Sodium		134 mmol/L	136mmol/L	138 mmol/L
Potassium		>6.0mmol/L	>6.0mmol/L 4.7 mmol/L	4.5mmol/L
Chloride		105mmol/L	106mmol/L	101 mmol/L



2D ECHO(19.12.24):

(N) LVSF

EF=58%

LA DILATED

NO RWMA

GOOD I LVDD(E/e'=11.5)

MILD AR(AR.P HT 809MS)

MILD TR (PASP-31MMHG)

MODERATE MR

POSTERIOR -MITRAL ANNULAR CALCIFICATION

NO CLOT /PE/PAH

CONDITION AT THE TIME OF DISCHARGE:**VITALS:**

PR - 92bpm

BP- 150/90mmHg

SpO2 -97%@RA

RR-18/min

S/E:

CVS : S1 S2+

RS : B/L NVBS

P/A : Soft non tender

CNS : NFND

COURSE IN THE HOSPITAL:

Patient initially admitted under surgery for AV Fistula, surgery done(13/12/24) and later on same day Night she developed sudden onset breathlessness and she was taken over by Medicine for Acute Pulmonary edema. Patient was started on Inj. Lasix IV 20mg BD and was given other supportive medication. On 14/12/24, Right femoral vein is cannulated with HD Catheter. Hyperkalaemia , correction given. 3 sessions of HD done @16/12/24 and 20/12/24. I/V/O Anemia(Hb-6.2 g/dl)-1 PRBC transfusion done along with HD(18/12/24). 2D Echo done, Cardio opinion obtained-advised T.Ecosprin AV 75/20 OD,T.Concor 2.5mg OD. Patient is now symptomatically better and hemodynamically stable and hence being discharged with the following advice.



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ADVICE ON DISCHARGE:
FRD/SRD/ DIABETIC DIET

S.NO	MEDICATION	DOSAGE	M	A	N	BEFORE FOOD	AFTER FOOD	No. Of Days
1	T.NICARDIA	10mg	1	1	1		✓	To continue
2	T.ECOSPIRIN AV	(75/20)	0	1	0		✓	To continue
3	T.CONCOR	2.5mg	1	0	0		✓	To continue
4	INJ.EPO	10,000 units	Weekly once s/c					To continue
5	T.AMARYL	1mg	1	0	1		✓	To continue

REVIEW IN NEPHROLOGY OPD IT SOS

NEXT HD @23/12/24

REVIEW IN MEDICINE OPD WITH FBS ,PPBS REPORTS.

In case of Emergency contact in the following numbers

Emergency No: [REDACTED] (8.00AM To 4.00PM)

Dr. G



D.,

DR. GENERAL MEDICINE
SLIMS

